

From Global Progress to Uneven Recovery

Over four decades, childhood vaccination grew from a patchwork of national programs into one of the most successful public-health efforts in history — reaching most of the world, but not all of it equally. COVID-19 then showed how fragile even decades of progress can be. The sections below walk through that story using our WHO/UNICEF coverage data (1980–2024), pairing explanatory text with interactive charts.

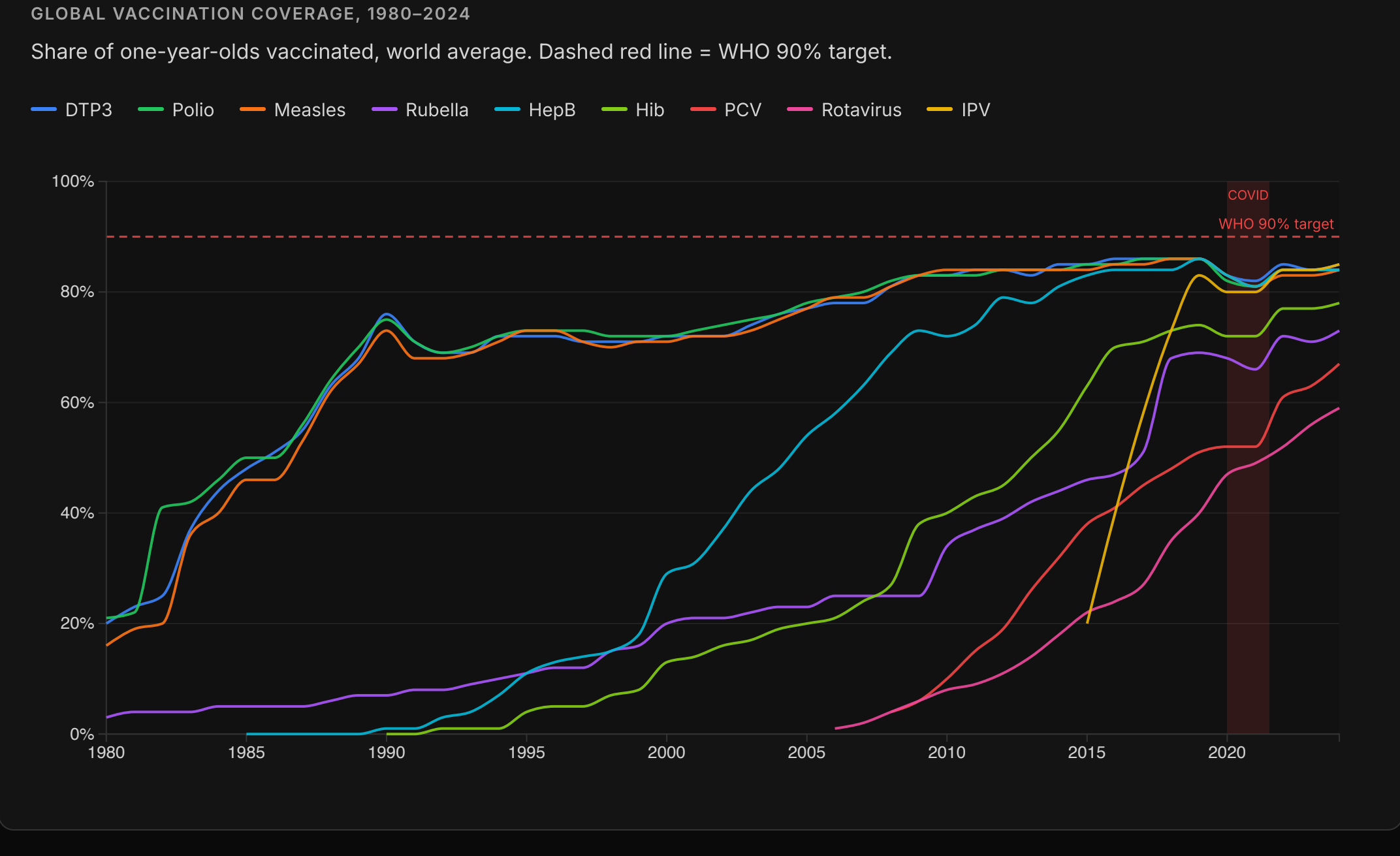
Communication Goals

- 1 **Trace the global rise:** Show how routine childhood vaccination coverage climbed from near zero to near universal, benchmarked against the WHO 90% target.
- 2 **Highlight geographic inequality:** Reveal how the world average hides dramatic gaps between countries and regions.
- 3 **Explore structural drivers:** Investigate whether wealth and development explain who vaccinates well — and who falls behind despite resources.
- 4 **Examine COVID disruption:** Measure how the pandemic reversed progress and which vaccines recovered fastest afterward.

LEVEL 1 · GLOBAL OVERVIEW

A Remarkable Rise in Global Coverage

From 1980 to 2024, routine childhood vaccination transformed from a privilege of wealthy nations into a near-universal standard. Every vaccine in the global schedule is shown; hover the chart to read off any year.

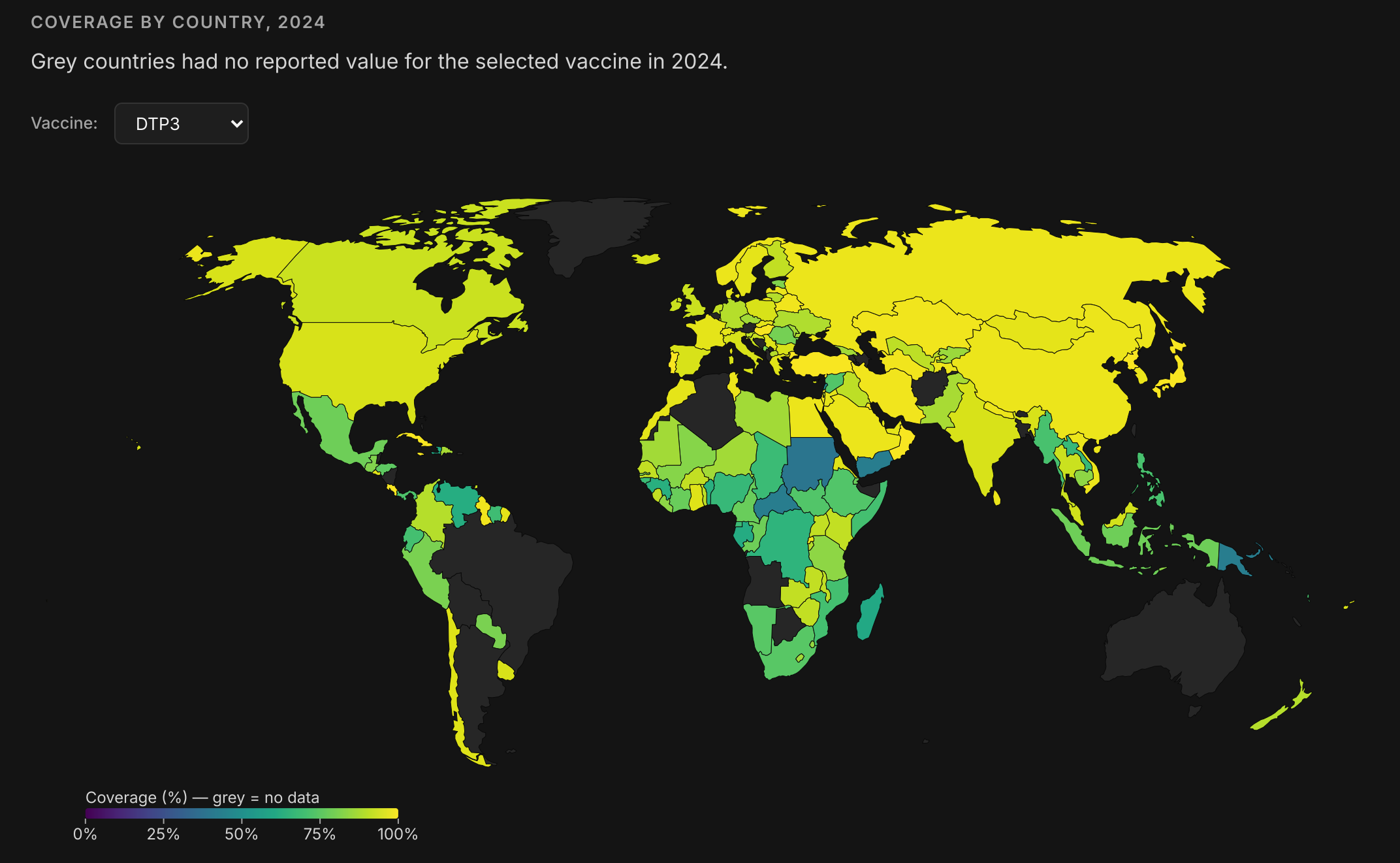


Key finding: DTP3 and Polio coverage rose from ~20% in 1980 to over 85% by 2019 — then dipped sharply during COVID. Newer vaccines (PCV, Rotavirus, IPV) climb from near zero only after their introduction into the routine schedule.

LEVEL 2 · GEOGRAPHIC INEQUALITY

The Global Average Hides a 60-Point Gap

The same 2024 average that suggests near-universal coverage masks dramatic inequality between countries. Pick a vaccine and hover any country to read its coverage.

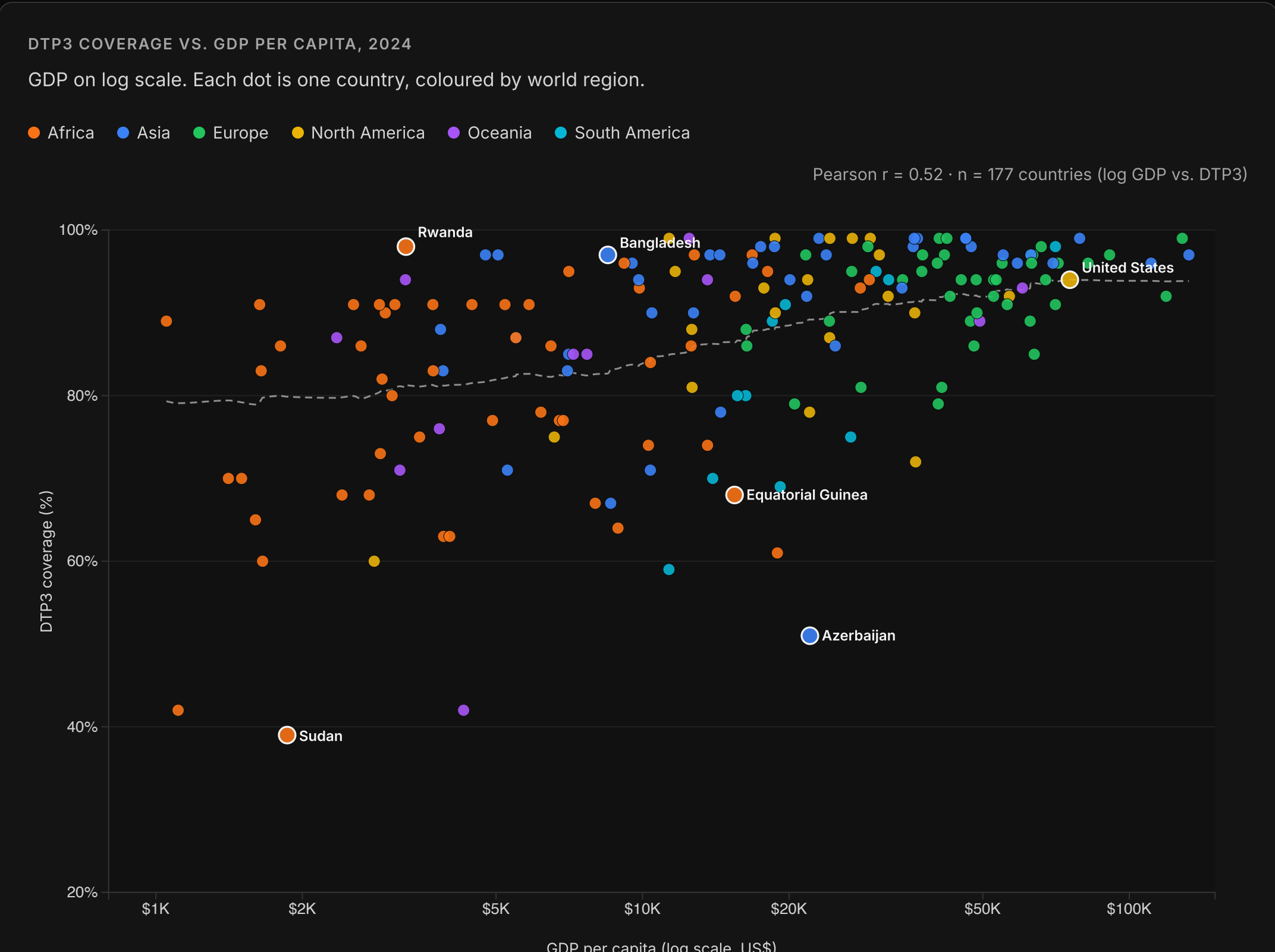


Key finding: Rwanda reaches 98% DTP3 coverage with a GDP per capita under \$3,500, while wealthier Azerbaijan sits at 51%. Coverage tracks public-health capacity more than national income — a point we return to in Level 3.

LEVEL 3 · WEALTH & COVERAGE

Money Matters — But Not the Whole Story

Coverage rises with GDP per capita, but the cloud is wide enough that nearly every income level contains both stars and laggards. The trend is real but loose.

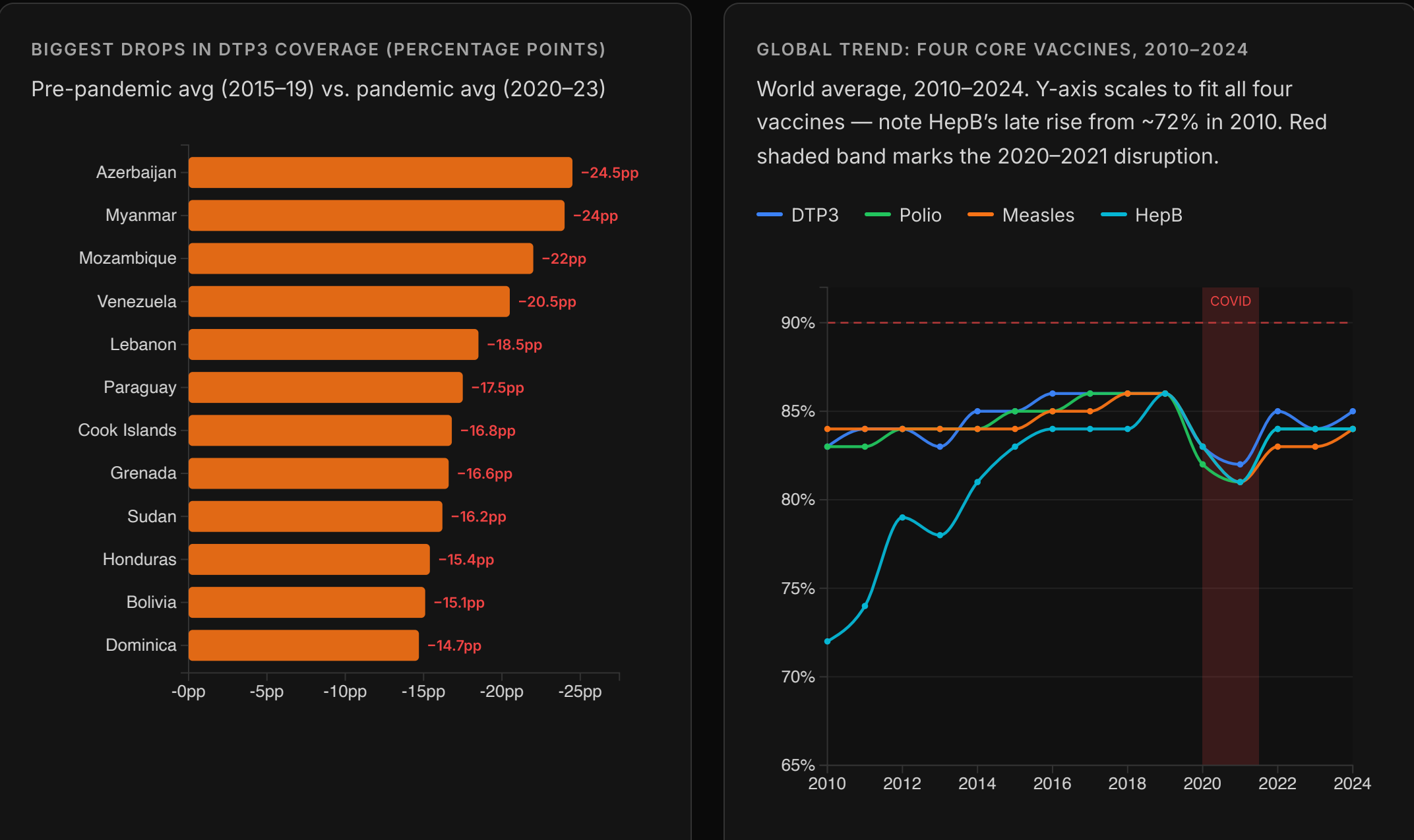


Key finding: Wealth and vaccination coverage move together — but only loosely. Richer countries tend to vaccinate more, yet Rwanda and Bangladesh outperform nations many times their income, while Azerbaijan lags far behind despite its resources. What matters most is a working primary-care system, not money alone.

LEVEL 4 · COVID DISRUPTION

The Pandemic Set Back Decades of Progress

Between 2019 and 2021, global DTP3 coverage fell from 86% to 82% — the first sustained decline since 1980. Some countries lost far more.



Key finding: Azerbaijan, Myanmar, and Mozambique each lost over 20 percentage points of DTP3 coverage during the pandemic. Globally, the system has nearly recovered (85% in 2024 vs. 86% in 2019) — but a single shock erased four years of progress, and measles, which needs ~95% coverage to prevent outbreaks, has been slowest to come back.